NAME: GOPAL RAO Y S	**AGE: 72** Year	**GENDER:** Male
SCAN ID: KNMP2508224	**REF Dr**: RAVI MOHAN	**DATE: 16**.08.2025

Department of PET-CT & Molecular Imaging

^{18}F-FDG PET – CT SCAN

Clinical history: **72 years old male with a history of ? glioma, was referred for PET CT evaluation.**

Technique: *10mCi of ^{18}F-FDG was injected intravenously to patient after 6 hours of fasting. After 60 min of injection, patient was scanned on dedicated* ***GE DISCOVERY IQ Generation 2 PET – CT Scanner****. PET images are reconstructed using advanced application* ***Q. clear****. Standard uptake values (SUV) normalized to body weight obtained over lesions. Blood glucose level at the time of injection was* ***102 mg/dl.***

Oral contrast *was administered for bowel opacification, over 90 min, before the scan. Oral contrast was given just before the scan as part of PET CT protocol on an* ***advanced high speed with IQE - CT scanner*** *with 1.25 mm slice thickness CE**CT** scan was acquired followed by PET acquisition and images were reconstructed on advanced* ***ADW 4.7®*** *workstation.*

Scanned area*: Head to mid-thigh*

Findings:

Brain:

- **FDG avid [SUVmax 11.3] heterogeneously enhancing lobulated soft tissue mass is noted along right peri- rolandic region involving posteriorly of right internal, right corona radiata, insular cortex, right putamen, right medial temporal lobe and contiguously extending into mid brain, measuring 2.4 x 2.7 x 2.6 cm on metabolica imaging. Note is made of mild perilesional edema with evidence of moderate obliteration of right ventricle.**
- Physiological tracer distribution is noted in the supra and infra-tentorial brain parenchyma. No SOL noted on CT images.

Note: All brain metastases may not be apparent on a PET/CT scan and MRI can be performed where clinically indicated.

Head & Neck:

- NO abnormal FDG uptake/soft tissue lesion is noted in the bilateral orbits/globes.
- Salivary glands demonstrate normal metabolic activity.
- The bilateral paranasal sinuses ,oral cavity, root and base of the tongue, nasopharynx, oropharynx, and hypopharynx show physiological FDG uptake with no morphological lesions on CT. The parapharyngeal spaces appear normal.
- Thyroid gland appears unremarkable with no demonstrable abnormal FDG uptake.
- ***Non-FDG avid subcentimeter sized bilateral level II and level III cervical lymph nodes are noted. Likely reactive***

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Thorax:

- Normal physiological FDG uptake is seen in the myocardium.
- Trachea and the main bronchi appear unremarkable.
- No evidence of any pleural or pericardial effusions seen.
- ***Non-FDG avid tiny pleural tags are noted in left upper lobe of the lung.***
- No focal abnormal FDG uptake is noted in the lung parenchyma or the pleura.
- No significantly enlarged or hypermetabolic mediastinal lymphadenopathy is noted.

Abdomen & pelvis:

- Liver appears normal in attenuation pattern, outline and shows physiological FDG uptake. No abnormal/ soft tissue lesion is noted in the liver parenchyma. IHBR and CBD appear normal in calibre.
- Stomach, small bowel and the large bowel loops appear unremarkable and reveal normal physiologic FDG uptake.
- Spleen measures within normal limits and reveals fairly homogeneous parenchyma & attenuation pattern with normal physiologic FDG uptake.
- Pancreas appears normal in size, shape & attenuation pattern with no demonstrable abnormal FDG uptake.
- Bilateral adrenal glands appear unremarkable with no abnormal FDG uptake.
- Bilateral kidneys appear normal in size, shape and attenuation pattern with normal physiologic FDG uptake.
- Urinary bladder is well distended and appears normal in shape & outline.

  Physiological tracer uptake is noted in the brain, myocardium, skeletal muscles, and cortex of bilateral kidneys with excretion of tracer into the urinary bladder.

Musculoskeletal:

- Normal physiologic FDG uptake is seen in the axial & proximal / distal appendicular skeleton under view.
- Normal physiologic tracer distribution / FDG uptake is noted in the muscles under view.

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IMPRESSION: Scan findings in this case reveals:

1. **FDG avid lobulated soft tissue mass is noted along right basal ganglia involving right insular and sub insular region with perilesional edema. Likely primary mitotic pathology.**
2. **No abnormal Hypermetabolism elsewhere in the body in the present study.**

Please correlate clinically
***Note**: All tumors are not FDG avid. In the absence of metabolically active disease reported on the scan, if there are other evidences to suggest presence of disease, complimentary investigations should be undertaken.*

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